1 Person-centred psychiatry perspectives on coercion and cooperation

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1.1 Introduction

The topic of coercive treatment in psychiatry is both complex and sensitive. It is complex as it involves, *inter alia*, clinical, public health, ethical and legal issues. And it is sensitive as it deals with delicate aspects of human experience and interpersonal relations. In order to offer a frame of reference to address such complexity and sensitivity, this chapter first summarizes an ongoing initiative on person-centred psychiatry and medicine. It then discusses the place for *dialogue*, partnership and trust in implementing person-centred care and dealing with the tension between social control and personal autonomy, between coercion and cooperation in health care.

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1.2 Psychiatry for the whole person

The basic thrust of person-centred clinical care is to place the person in context at the centre of health care. It involves shifting the focus of the field from disease to patient to person [1,2].

1.2.1 Origins and early developments

The roots of person-centred care can be traced back to ancient civilizations both Eastern (e.g. Chinese and Ayurvedic) and Western (principally Hellenistic) which employed a broad concept of health and organized treatments in a personalized manner. Such a broad concept of health (full wellbeing and not only the absence of disease) has been incorporated into the World Health Organization (WHO) definition of health.

In response to disease-based distortions in modern medicine [3], pioneering developments on person-centred perspectives emerged in the twentieth century, illustrated by Tournier's *Medicine de la Personne* in Switzerland [4], Rogers' person-centred approach focused on open communication and empowerment in the US [5], McWhinney's family medicine movement in the UK and Canada [6], Brera's person-centred medicine programme in Italy [7], and Alanen's need-adaptive assessment and treatment approach in Finland [8]. There has been, as well, increasing recognition of the key role of a collaborative clinician–patient relationship. Tasman pointed out in his American Psychiatric Association presidential address [9] that this relationship must start from the first encounter and represents the fundamental matrix for the whole of care. It must ensure empathetic listening, comprehensive diagnosis beyond symptom checklists, appreciation for symbolic meaning, and effective therapeutic partnerships instead of narrow and reductionistic approaches.

In a related development, *recovery-orientated* concepts emerged from a coalescence of efforts from both mental health service users and professional groups. On one hand, individuals who suffered and recovered from mental illnesses formed a recovery movement and a national and international community of activists (e.g. [10,11]) who expressed their criticism against negatively experienced psychiatric treatment, and demanded to be considered active protagonists and partners rather than passive recipients of care. On the other hand, well-known experts in the field of psychosocial rehabilitation (e.g. [12]) have welcomed the statements of service users and the need to attend to their subjective experiences, in contrast to traditional rehabilitation which appeared focused on improving functioning and adaptation and not on the fulfilment of the individual human being. At least since the beginning of the 1990s, many professionals in rehabilitation and other mental health fields, and various service user groups have started substantial collaboration through joint publications and conferences [13].

Also public health policy developments have been moving in a person- and people-centred direction. In a landmark study [14], the Institute of Medicine in the United States concluded that the American health system was seriously flawed and required a new framework with particular aims and rules, a key one being person-centeredness. Along the same lines, a US Presidential Commission on Mental Health diagnosed a state of disarray in national mental health care and proposed a thorough transformation to be driven by the patient and the community in a recovery orientation manner [15]. Likewise, the World Health Organization European Office [16] held a ministerial event which concluded that integration of mental health services should be organized by the involvement of patients and caregivers; that is, collectively tackling stigma, discrimination and inequality, and empowering and supporting people with mental health problems and their families to be actively engaged in this process; furthermore, recognizing the experience and knowledge of service users and caregivers as an important basis for planning and developing services.

Most recently, the WHO World Health Assembly formulated innovative resolutions for primary health care [17], two pillars of which are people-centred care and health systems responsiveness to human and social aspirations.

1.2.2 Emergence of person-centred psychiatry and medicine

As elucidated by historians Garrabe and Hoff from the proceedings of the First World Congress of Psychiatry of September 1950 [18], the World Psychiatric Association (WPA) revealed since its inception a keen interest on articulating science and humanism as the seeds of a person-centred psychiatry. But it was only in 2005 that the WPA General Assembly established an Institutional Program on Psychiatry for the Person: from Clinical Care to Public Health (IPPP). This initiative affirmed the *whole person in context* as the centre and goal of clinical care and health promotion, at both individual and community levels. It pointedly argued for the articulation of the scientific method and humanism to optimize attention to the ill and the positive aspects of a person's health, the integration of all relevant health and social services, and the advancement of pertinent public health policies.

The person-centred approach promotes clinical care *of* the person (the totality of the person's health, including its ill and positive aspects), *for* the person (to assist in the fulfilment of the person's life project), *by* the person (with clinicians extending themselves as full human beings, scientifically grounded and with high ethical aspirations), and *with* the person (in respectful and empowering collaboration with persons who present for care). The person is conceptualized with its full pluralistic richness and within its life context in line with Ortega y Gasset's dictum *I am I and my circumstance*. This approach represents a paradigmatic shift from the disease to the person as the main target and centre of medicine and health care [19,20].

Amongst the key elements of person-centred care are the following: (1) A broad biological, psychological, socio-cultural and spiritual framework. (2) Attention to both ill health and positive health. (3) Person-centred research on the process and outcome of clinician–patient–family communication, diagnosis, clinical care and health promotion. (4) Promotion of and respect for the autonomy, responsibility and dignity of every person involved.

Person-centeredness, in addition to its ethical relevance, may be shown to enhance the effectiveness and outcome of the various aspects of clinical care, from interviewing the person who consults and pertinent family members, to comprehensive diagnosis, and to the various aspects of treatment and health promotion. Illustratively, person-centred diagnosis [21,22] is primarily aimed at optimizing treatment planning, and involves pointed attention to health status, to experience of health and illness and to contributory agents including risk and protective factors, and it employs descriptive categories, dimensions and narratives, and activates the interaction of clinicians, the patient and family members. A perspective has been articulated recently to integrate conventional disease-centred or nosological diagnosis within a broad, person-centred diagnosis [23].

Person-centred medicine training programmes are emerging in several countries, such as those at Ambrosiana University Medical School in Milan, Italy, Birmingham University in the UK (A. Miles, Medical School Dean, personal information, 1 December 2009), and at Cayetano Heredia Peruvian University in Lima (J. Saavedra, Professor of Psychiatry, personal information, 18 December 2009). Curricula are being developed at the listed medical schools and through the International Network for Person-centered Medicine (INPCM) [24]. Additionally to be noted are Conferences on Psychiatry for the Person in London (co-organized by the WPA Institutional Program on Psychiatry for the Person (IPPP) and the UK Department of Health), October 2007, Paris (co-organized by the WPA IPPP and the Association of WPA French Member Societies) February 2008, and Prague (WPA XIV Congress of Psychiatry), September 2008, and the Geneva Conferences on Person-Centered Medicine in May 2008 and 2009 [25].

The person-centred medicine academic programmes and events listed above pointedly involve also research activities. These include both conceptual explorations [1,26] as well as the presentation of empirical research on topics such as clinical communication [27,28] and Person-Centred Integrative Diagnosis [21,22].

1.3 The WPA Dresden conference – reviewing coercion and cooperation

A key element of the above outlined initiative is to promote a psychiatry and medicine *with* the person; that is, the affirmation of the personhood of the patient and

the commitment to work in respectful and collaborative partnership with the person who consults. This includes, first, work with individuals highlighting the ethical underpinnings of this effort. It also encompasses work with patient groups including those critical of psychiatry.

It is on these grounds that the WPA Thematic Conference held in Dresden on 6–8 June 2007 represented a crucial opening for dialogue. The conference had an intriguing and sensitive overall topic, *Coercive Treatment in Psychiatry: A Comprehensive Review*. Professor John Monahan, Scientific Committee chair, anticipated on his invitation letter that

> while the fissures in this area run so deep and are so long-standing that achieving consensus is unlikely, our aspiration is that this historic meeting will sharpen moral issues, clarify political viewpoints, identify evidence-based practices, and share cutting-edge data on one of the most contested topics of our time.

In fact, as reported by Professor Thomas Kallert, Organizing Committee chair, the Conference succeeded in attracting participants from 36 different countries, with virtually all world experts on this field attending and speaking at it, all leading to an absolutely top-quality scientific programme.

But there was, additionally, a surprising event that marked the Dresden Conference indelibly for all its organizational and individual participants. Many of the user groups critical of psychiatry (but not all), which traditionally would be expected to protest outside, decided to come in and engage in a discussion of serious concerns. This opening had a crucial value for global psychiatry as this substantially broadened the range of its patients/users interlocutors which also encompass groups (including self-help groups) with which psychiatric organizations have been interacting for a long time.

In a historic encounter on 6 June, requested formally and with the endorsement of the WHO by Mind Freedom International, the European Network of (ex-)Users and Survivors of Psychiatry and world networks of current and past users of psychiatric services (ENUSP, World Network of Users and Survivors of Psychiatry WNUSP), the president and other top leaders of the WPA met with four representatives of the user organizations. The encounter, originally scheduled to last one hour, spontaneously extended to three. A range of issues were discussed, and possibilities for continuing the dialogue in congresses and other settings were explored. As reported by David Oaks [29], Director of Mind Freedom International, 'This conversation was different than usual. Yes, once more, the proof will be in the results. But all involved felt they were heard and respected in this discussion'.

During the following day, the WPA Executive Committee temporarily suspended its official business meeting in order to attend the keynote lecture by Ms Dorothea Buck on '70 Years of Coercion in German Psychiatric Institutions, Experienced and

Figure 1.1 Seated Ms D. Buck; standing (left to right): Professors H. Herrman, S. Tyano, M. Amering, J. E. Mezzich, M. Jorge, J. Cox and P. Ruiz at the WPA Thematic Conference, Dresden, 7 June 2007.

Witnessed'. On the basis of her personal history, she challenged a psychiatry that neglects communication with patients, and demanded a paradigm shift based on the wealth of patients' experiences. After her lecture, the WPA president offered a thank-you speech for Ms Buck's articulate and moving lecture, and greeted her in the company of all present members of the WPA Executive Committee and the session chair (see Figure 1.1).

At an immediately ensuing press conference, representatives of the WPA, Council of Europe, and user organizations sitting at the main table held a lively exchange of questions, answers and comments with press representatives and the general audience (Figure 1.2). The issues experienced globally by service users, the patterns and diversity of their organizations, and prospective opportunities for continuing the Dresden dialogue and for user participation in activities of global and national psychiatry activities were broadly discussed.

1.4 Evolving developments for enhancing cooperation

The current volume on *Coercive Treatment in Psychiatry* represents a principal follow-up of the 2007 Dresden Conference. Many of its speakers, including multidisciplinary scholars and users, are present again through its pages. The topics covered comprise a comprehensive clinical, public health, ethics and legal review of

Figure 1.2 (Left to right): Professor T. Kallert (conference organizer), Dr V. Pimenoff (Council of Europe representative), Mr P. Lehmann (author and German User Network representative), Professor J. E. Mezzich (president of the WPA and of the conference), Mr D. Oaks (Mind Freedom International Director) and Professor J. Cox (WPA Secretary General), seated at the press conference of the WPA Thematic Conference in Dresden, 7 June 2007.

the volume's complex and sensitive theme. As a group they encompass both intellectual and experiential, scientific and humanistic perspectives. They can be conceived, therefore, as representing a *person-centred* approach.

Some steps have been undertaken in the years that followed Dresden in terms of invitations to user and family representatives to speak at person-centred conferences in Paris (February 2008), Prague (September 2008) and Geneva (May of 2008, 2009 and 2010). Joint studies involving users, families and professionals are evolving as well [13,30]. The room for further collaborative work, however, is ample indeed.

Additionally, one can list some recent innovative developments that may contribute directly or indirectly to addressing usefully and helpfully the tension between coercion and cooperation. They refer in various degrees to clinical care and public health.

One such development is the ongoing preparation of a Person-Centred Integrative Diagnosis model undertaken by the INPCM [24]. This comprehensive diagnosis model covers both ill health, through a nosological diagnosis component based on the WHO International Classification of Diseases, and positive health (self-awareness, resilience, sense of control and transcendence, resources, well-being), attends to the person's experience and values as well as to risk and protective health factors, employs categories, dimensions and narratives as descriptive instruments, and engages the patient, family and clinicians in partnerships for shared diagnostic understanding and shared commitment to care [22]. Extending the practical prospects for person-centred diagnosis, a number of national and regional developments incorporating such approaches are being developed in Europe [31] and Latin America [32].

Based on the resolutions of the WHO [17] World Health Assembly promoting people-centred primary health care, the WHO and the INPCM are planning the

Fourth Geneva Conference on Person-Centered Medicine for 2–4 May 2011, which will include presentation of an evolving study on the systematic conceptualization and measurement of person- and people-centred care. This may open new horizons for addressing control–cooperation issues in a diversity of countries.

Applicable to all aspects of health care are research and development efforts towards enhancing clinical communication. They facilitate direct acquaintance, fluid exchange of information, and presentation of helpful intentions and attitudes [27,33]. As such, they can powerfully foster empathy and trust, crucial elements of person-centred care and relevant to understanding and addressing usefully the tension between coercion and cooperation.

1.5 Concluding remarks

We have briefly reviewed broad historical and contemporary perspectives on person-centred care, and global psychiatry and medicine's evolving scientific and humanistic response to them. A renewed commitment to the clinician–patient relationship appears crucial, as well as building an effective dialogue with service users and families, respecting the diversity of their perspectives. Let's take advantage of the Dresden opening and developments that followed to find creative paths to jointly address specific coercion–cooperation issues and cultivate a shared commitment to promote health in individuals and communities across the world.

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